

← Right-Sided Heart Failure ←

← Condition/Cause ←	← Treatment ←	← Monitor ←
When the tricuspid valve doesn't close properly, some blood leaks back into the upper right chamber (atrium) instead of going to the lungs, then goes back down the veins. ← Symptoms ← 1. Weight gain due to peripheral edema: Swelling, especially in ankles and legs, caused by fluid retention in tissues 2. JVD (Jugular vein distention): Neck veins become more visible when lying flat 3. Abdominal swelling (ascites): Fluid accumulation in the abdominal cavity, causing tightness and discomfort 4. Shortness of breath: Even at rest 5. Liver enlargement (Hepatomegaly) 6. Swollen spleen: In some cases 7. Pitting edema: Indentation remains briefly when pressing swollen areas like ankles 8. High blood pressure 9. Tachycardia: Rapid heart rate as the heart tries to compensate for decreased efficiency	1. Lasix (furosemide): This is a loop diuretic that helps the body get rid of excess fluid, but it can lead to potassium loss, so potassium levels should be monitored. 2. Beta Blockers and Calcium Channel Blockers: These medications help reduce the heart rate, easing the workload on the heart. However, if the heart rate drops too low (less than 60 beats per minute for adults, less than 70 for children, and less than 90 for infants), these medications should be STOPPED. Same with digoxin. 3. Digoxin: This medication also helps reduce the heart rate and can be used alongside beta blockers and calcium channel blockers, but its dosage needs careful monitoring. Note All heart failure patients should be positioned in a semi-Fowler's position, which is an angle of 30 to 45 degrees. The first thing RN should do when entering the patient's room is to check their position, with 45 degrees being the ideal angle for these patients.	Heart Rate: Regular monitoring of heart rate is crucial to adjust medication dosage. Potassium Levels: Since Lasix can deplete potassium, it's important to keep an eye on potassium levels and supplement if necessary. ← Complications ← Cor Pulmonale is COPD + RS heart failure! Cor Pulmonale is right-sided heart failure caused by high blood pressure in the pulmonary arteries (pulmonary hypertension), often due to chronic lung diseases like Chronic Obstructive Pulmonary Disease (COPD). This results in long-term pressure overload on the right side of the heart. Note Loop Diuretics Lower Potassium in blood Digoxin toxicity - Dehydration can lead to Digoxin toxicity. Digoxin with furosemide is a CAUTION. Early signs are nausea and vomiting. Visual disturbances - Halo over the objects.

➡ Left-Sided Heart Failure ➡		
➡ Condition/Cause ➡	➡ Treatment ➡	➡ Monitor ➡
When the mitral valve doesn't close properly, some blood leaks back into the left atrium from the left ventricle instead of going into the aorta. This backflow of blood causes congestion and increased pressure in the lungs, as some of the blood returns to the pulmonary circulation.	1. Diuretics: To reduce fluid buildup in the body 2. ACE Inhibitors or ARBs: To lower blood pressure and reduce strain on the heart 3. Aldosterone Antagonists: To help remove excess sodium and water from the body. 4. Digoxin: To strengthen heart contractions and slow the heart rate (use with caution). Medications to Avoid: 1. Beta Blockers: DO NOT give any kind of Beta Blockers due to the risk of bronchospasm as a side effect, especially in patients with lung problems. 2. Certain Calcium Channel Blockers: Avoid Diltiazem and Verapamil because they can worsen heart failure symptoms and may cause bronchospasm.	Oxygen Levels: Regularly check blood oxygen saturation Blood Pressure: Keep track to manage hypertension
➡ Symptoms ➡		➡ Complications ➡
1. Pulmonary Edema: Fluid buildup in lungs 2. Shortness of Breath: Due to fluid buildup 3. Dyspnea: General difficulty in breathing 4. Crackles: Abnormal lung sounds 5. Rhonchi: Rattling lung sounds 6. Hypoxia: Low oxygen levels in the blood 7. Chest Pain: Discomfort or pain 8. Fatigue: Extreme tiredness and reduced ability to perform physical activities 9. Hypertension: High blood pressure	Note All heart failure patients should be positioned in a semi-Fowler's position, which is an angle of 30 to 45 degrees. The first thing RN should do when entering the patient's room is to check their position, with 45 degrees being the ideal angle for these patients.	